THE Open GROUP

TOGAF® Standard – Implementation and Migration Plan

Version 1.0

October 2026

Barangay San Antonio Health Department Enterprise Architecture

Barangay San Antonio Health Center, Pasig City

Document Version History

Version Number	Version Date	Revised By	Description	Filename
1.0	October 22, 2026	UA&P Solution Architecture team	First issue, based on interviews (October 2–21, 2026) and verification answers (October 22, 2026)	E4 - Implementation and Migration Plan.docx

Contents

1. Purpose of this Document 5
   1.1 Output and Input 5
2. Implementation and Migration Strategy 6
   2.1 Strategic Implementation Direction 6
   2.2 Implementation Sequencing Approach 6
3. Project and Portfolio Breakdown of Implementation 7
   3.1 Allocation of Work Packages to Project and Portfolio 7
   3.2 Capabilities Delivered by Projects 7
   3.3 Milestones and Timing 7
   3.4 Work Breakdown Structure 7
   3.5 May Include Impact on Existing Portfolio, Program, and Projects 8
4. Project Charters 9
   4.1 Included Work Packages 9
   4.2 Business Value 9
   4.3 Risk, Issues, Assumptions, Dependencies 9
   4.4 Resource Requirements and Costs 9
   4.5 Benefits of Migration, Determined (Including Mapping to Business Requirements) 10
   4.6 Estimated Costs of Migration Options 10

A Abbreviations 11

Tracking Information

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Title	Implementation and Migration Plan	**Document Version Date**	October 22, 2026
Reviewed By	Dr. Glynnis Lynn U. Bonilla, Medical Officer IV (Executive Sponsor)	**Review Date**	October 23, 2026

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1. Purpose of this Document

The Implementation and Migration Plan provides a schedule of the projects that will realize the Target Architecture. The Implementation and Migration Plan includes executable projects grouped into managed portfolios and programs. The Implementation and Migration Strategy identifying the approach to change is a key element of the Implementation and Migration Plan.

1.1 Output and Input

The TOGAF® Content Framework (see Architecture Deliverables) identifies deliverables that are produced as outputs from executing the Architecture development Method (ADM) cycle and potentially consumed as inputs at other points in the ADM.

This deliverable is produced, updated, and consumed by the ADM Phases:

Deliverable	Output from…	Input to…
Implementation and Migration Plan	E, F	F

2. Implementation and Migration Strategy

2.1 Strategic Implementation Direction

Start with what the barangay and health center control and can fund quickly, use free or low-cost tools, keep the City EMR as the system of record, and build an evidence-based case for City-level changes in parallel.

2.2 Implementation Sequencing Approach

- Quick wins first (November–December 2026), timed to the barangay budget deadline (November 2026).
- No rollouts during OPT Plus and MR-SIA (January–March) or month-end reporting.
- Equipment and digitization in April–June 2027, after the field season.
- Integration and extension in July–December 2027, depending on City responses.

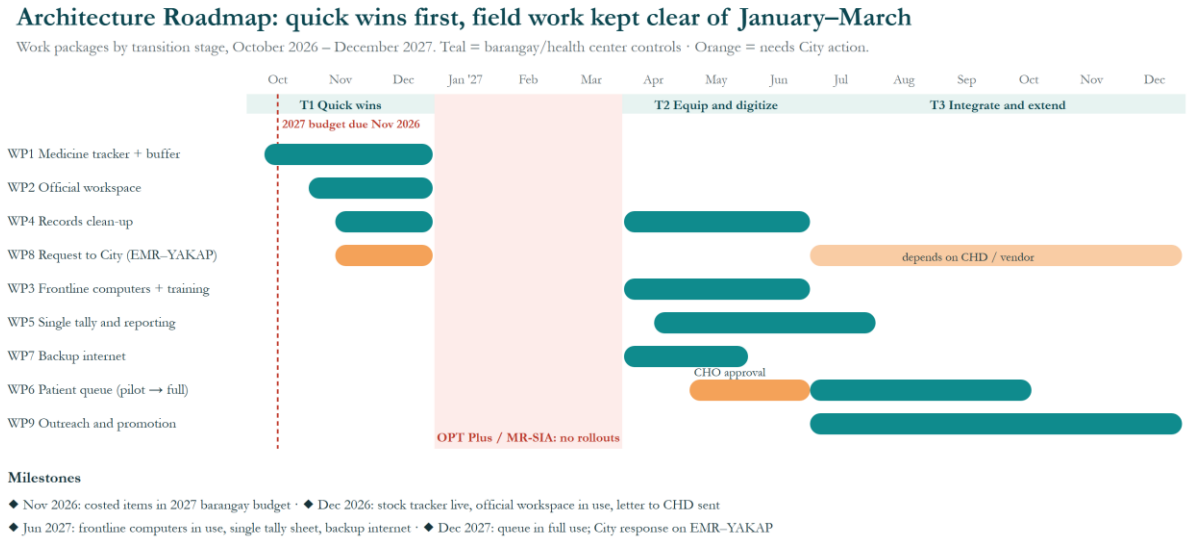


Figure 1. Architecture Roadmap.

What this shows: when each improvement (work packages WP1–WP9) happens between October 2026 and December 2027. Teal bars are things the barangay or health center controls; orange bars need City action. Nothing new is rolled out from January to March, when staff are busy with OPT Plus and MR-SIA field work. **Why it matters:** quick, low-cost wins come first, timed to the 2027 barangay budget due in November 2026.

3. Project and Portfolio Breakdown of Implementation

3.1 Allocation of Work Packages to Project and Portfolio

Project	Work packages	Sponsor
P1 Medicine Assurance	WP1	Kagawad Rustia; Nurse Hardman
P2 Digital Workplace	WP2, WP3, WP5, WP7	Kapitan Lising
P3 Records Modernization	WP4	Dr. Bonilla
P4 Patient Flow	WP6	Dr. Bonilla; UA&P IT program
P5 City Interoperability	WP8	Dr. Bonilla; Kapitan Lising
P6 Community Reach	WP9	Kagawad Rustia

3.2 Capabilities Delivered by Projects

Project	Capability improved (baseline → target)
P1	Medicine and supply logistics 2 → 4
P2	IT and equipment 2 → 4; Reporting and compliance 3 → 4
P3	Records management 2 → 4
P4	Patient care 3 → 4
P5	Records management and patient care (City-wide)
P6	Community outreach 3 → 4

3.3 Milestones and Timing

Date	Milestone
November 2026	Costed items (WP1 buffer, WP3, WP7, WP9) proposed in the 2027 barangay budget
December 2026	Stock tracker live (WP1); official workspace in use (WP2); active-chart index started (WP4); letter to the City Health Department sent (WP8)
January–March 2027	No rollouts (OPT Plus and MR-SIA field work)
June 2027	Frontline computers in use (WP3); single tally sheet (WP5); backup internet (WP7); queue pilot running after CHO approval (WP6)
December 2027	Queue in full use (WP6); outreach expanded (WP9); City response on EMR–YAKAP integration (WP8)

3.4 Work Breakdown Structure

Project	Main tasks
P1	Design stock sheet with reorder levels → load current balances → train nurse and aides → reconcile monthly → propose buffer line
P2	Set up barangay account and shared drive → migrate program files → work-only chat groups → buy and install computers → train aides and BHWs → build tally sheet → install backup line
P3	Agree filing standard → build active-chart index → box and label inactive charts → schedule disposal → install cabinet and key log
P4	Seek CHO approval → define queue rules → UA&P capstone build → pilot on consultation days → review
P5	Compile evidence (retyping time, examples) → letter via Nurse Hardman → follow up quarterly

Project	Main tasks
P6	Plan Wellness Pop-up calendar with condo admins → oral health campaign → equip BNS for field entry

3.5 May Include Impact on Existing Portfolio, Program, and Projects

No conflict with City programs. P4 aligns with Dr. Bonilla's goal of a queuing system and the UA&P IT capstone pipeline. P6 builds on the existing Community Wellness Pop-ups.

4. Project Charters

4.1 Included Work Packages

Project	Work package	Description
P1	WP1 Medicine stock tracker and buffer stock	Shared stock sheet (beginning balance, received, issued, ending balance, reorder level) updated daily; standing barangay buffer line for maintenance medicines (Metformin, Losartan, Amlodipine ran short in 2026)
P2	WP2 Official workspace and communication	Barangay-owned Google account or Workspace with a shared drive for program files; work-only chat groups; storage for OPT files
P2	WP3 Frontline computers and digital skills	2–3 shared desktops for the 4 Pasig Health Aides and 3 BHWs; basic computer workshops (UA&P outreach)
P3	WP4 Records clean-up and security	Active-chart index by record number for the ~8,000 charts; archive inactive charts on the DOH 7-year schedule; lockable records cabinet and key log; request consent once per patient (CHD approval)
P2	WP5 Single tally and reporting	Aides and BHWs enter daily tallies once on the shared computer; program reports built from that sheet; request CHD to accept digitally signed reports
P4	WP6 Patient queue	Numbered tickets and display for consultation days; built as a UA&P IT capstone pilot after City Health Officer approval
P2	WP7 Backup connectivity	Second provider or LTE router with failover alongside the PLDT line; written outage procedure (paper fallback)
P5	WP8 City interoperability request	Evidence-based letter to CHD: EMR–PhilHealth YAKAP prescription link, dental eMED link, replacement of 10-year-old City laptops, digital report submission
P6	WP9 Outreach and health promotion	Condo partnerships through the Community Wellness Pop-ups; oral health campaigns; BNS field laptop and complete weighing kit

4.2 Business Value

No medicine stock-outs, less retyping, faster records, protected work data, fairer waiting and wider reach (details in the Architecture Roadmap, Business Value).

4.3 Risk, Issues, Assumptions, Dependencies

See the Architecture Roadmap, Implementation Factor Catalog. The key dependency is inclusion of costed items in the 2027 barangay budget (proposals due November 2026).

4.4 Resource Requirements and Costs

Work package	Resources	Indicative cost
WP1 Medicine stock tracker and buffer stock	Nurse Hardman; Kagawad Rustia	Tracker ₱0; buffer about ₱150,000–200,000 a year (2026 spend: about ₱150,000)
WP2 Official workspace and communication	Barangay IT; Kagawad Rustia	₱0 (shared barangay account) to about ₱4,000–5,000 a month (paid workspace, about 10 users)
WP3 Frontline computers and digital skills	Kapitan Lising; UA&P team	About ₱50,000–105,000 for 2–3 desktops; training ₱0

Work package	Resources	Indicative cost
WP4 Records clean-up and security	Ms. Roa (custodian); Ms. Abong	Cabinet about ₱10,000–20,000; staff time
WP5 Single tally and reporting	Ms. Abong; Nurse Hardman	₱0 (templates)
WP6 Patient queue	Dr. Bonilla; UA&P IT program	Development ₱0 (capstone); ticket printer and display about ₱15,000–30,000
WP7 Backup connectivity	Barangay IT	About ₱1,500–2,500 a month plus router
WP8 City interoperability request	Dr. Bonilla; Kapitan Lising	₱0 to the barangay
WP9 Outreach and health promotion	Kagawad Rustia; Dr. Magpali; Mr. Vasquez	Field laptop about ₱30,000; weighing kit to be quoted

Costs are indicative estimates from typical 2026 Philippine retail prices, not quotations; they must be verified through barangay procurement.

Indicative total: one-time about ₱105,000–185,000 (computers, cabinet, queue hardware, field laptop) plus about ₱150,000–200,000 a year for the medicine buffer and about ₱18,000–90,000 a year for connectivity and workspace, depending on options chosen.

4.5 Benefits of Migration, Determined (Including Mapping to Business Requirements)

Benefit	Requirement	Measure
Medicines on hand	R-02, R-03	SM-2: stock-out months → 0
One prescription entry (if City adopts)	R-01	SM-1: 3 → 1
Faster records	R-09, R-10	SM-3: < 1 minute
Reports in working hours	R-07	SM-4: none after hours
Protected work data	R-05	SM-6: none on personal accounts
Frontline staff equipped	R-04, R-11	SM-7: all with access
Fairer queue	R-06	Fewer complaints
Consultations through outages	R-08	No paper-only days

4.6 Estimated Costs of Migration Options

Decision	Option A (lowest cost)	Option B (stronger)
Workspace	Free shared barangay Google account and drive	Paid Google Workspace (~₱4,000–5,000 a month, ~10 users)
Backup internet	Prepaid LTE router (~₱1,500 a month)	Second fiber provider (~₱2,500 a month)
Frontline computers	2 desktops (~₱50,000–70,000)	3 desktops (~₱75,000–105,000)
Queue	Paper numbered tickets + whiteboard (~₱0)	UA&P capstone app with display (~₱15,000–30,000 hardware)

Costs are indicative estimates from typical 2026 Philippine retail prices, not quotations; they must be verified through barangay procurement.

A Abbreviations

Abbreviations, acronyms and reference codes used in this document and its figures, in alphabetical order.

Abbreviation	Meaning
ADM	Architecture Development Method, the step-by-step method of the TOGAF Standard
BHW	Barangay Health Worker
BNS	Barangay Nutrition Scholar
CHD	City Health Department (Pasig City)
CHO	City Health Officer
DOH	Department of Health
eMed / eMED	Name of the City's electronic medical record system; “dental eMED” is its separate dental system
EMR	Electronic Medical Record
Hon.	Honorable (title of elected officials)
IT	Information technology
LTE	Long-Term Evolution (4G mobile internet)
MR-SIA	Measles-Rubella Supplemental Immunization Activity
OPT / OPT Plus	Operation Timbang Plus: yearly weighing and measuring of children aged 0–59 months
PhilHealth	Philippine Health Insurance Corporation
PLDT	PLDT Inc., an internet and telephone provider
R-01 …	Stakeholder requirement reference number
SM-1 …	Success measure reference number
T1 … T3	Transition architecture stage
TOGAF	The Open Group Architecture Framework (the TOGAF® Standard)
UA&P	University of Asia and the Pacific
WP1 … WP9	Work package (a group of related changes delivered together)
YAKAP	PhilHealth Yaman ng Kalusugan Program (primary-care benefit)